

Reader: a friend who already knows Evan.

Tunza

The child who waits

Evan's starting point is specific: eCHIS does not order which sick child goes first. For the child left waiting, the unanswered question is who needs attention now. [1]

The care path can then fail at the decision, destination, facility, travel, acknowledgement, or returned-result step. Tunza has to exist as an attempt to solve that gap. The public demo does not do this yet. It does not order that queue. [2]

People already die inside this system's gaps. The [WHO Kenya 2025](#) report puts maternal mortality at an estimated 355 deaths per 100,000 live births, from preventable causes. Kenya's Ministry of Health names a fragmented referral system and says strengthening it aims at maternal and child health outcomes. Those are the country's numbers. They are not Tunza's results, and they are not claims about the demo. [8]

The ambition is to follow a referral far enough to know whether the next step was right. Creating a referral is not the same as knowing someone received care. [3]

What you can try today

The public demo lets you walk through a case, with household, community health worker, and facility views. The words on screen are in English and Kiswahili. [4]

You can describe what is happening in text and answer short questions about the person's condition. Demonstration rules produce a next-step category: go now, get care today, monitor at home, or answer another question. You can also walk through referral stages and record a local outcome, including "did not arrive" or "unknown." [5]

The limits are concrete. It is not a doctor or a diagnosis. The rules are not clinically validated. Cases stay in the browser on that device, and the referral facilities are fake demo facilities. It is not a substitute for emergency services or professional medical care. [6]

What he still does not know

These questions remain unanswered. [7]

- **Q1 — What counts as the right decision?** Which outcome would become the answer a future system learns from, who would record it, and when?
- **Q2 — How does it start?** With no labeled outcomes, what would the first version rank cases on?
- **Q3 — Who can use the data?** Access to eCHIS would not itself grant permission to train on its data.
- **Q4 — When do alerts stop being useful?** How many false urgent alerts would a community health worker tolerate before ignoring the system?
- **Q5 — Is this one product or two?** Ordering cases for a health worker and helping a household decide what to do next are different jobs.

The need to decide who goes first is the reason to build. It is not evidence that the demo can make that decision.

Sources

[1] ../tunza-vision-2026-09-23/fact-sheet.md:72-72. [2] fact-sheet-urgent.md:11-17, ../tunza-vision-2026-09-23/fact-sheet.md:72-74. "Has to exist" expresses intent, not a measured effect. [3] ../tunza-vision-2026-09-23/fact-sheet.md:57-58. [4] ../tunza-vision-2026-09-23/fact-sheet.md:22-25. [5] ../tunza-vision-2026-09-23/fact-sheet.md:27-32,36-41. [6] ../tunza-vision-2026-09-23/fact-sheet.md:31-39. [7] ../tunza-vision-2026-09-23/fact-sheet.md:81-85. [8] fact-sheet-newdoc.md S13-S14 (WHO Kenya Annual Report 2025, printed p. 60; MoH referral-policy page, Mar 2026).